INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Carboplatin · 21-day breast schedule

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00261
Version 7

21-day cycle

Treatment stages

01 **Carboplatin · 21-day breast schedule**
21-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology emergency service immediately for fever or shaking chills, sudden illness, bleeding, new breathlessness, chest pain or severe allergy. Report reduced urine, hearing changes and new numbness promptly. Do not wait for the next visit.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

Aim to slow or shrink metastatic breast cancer and help symptoms, balancing possible benefit, side effects and treatment burden.

PLATINUM CHEMOTHERAPY

Carboplatin

intravenous (IV) infusion



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) through cross-links

Carboplatin forms links in DNA that interfere with DNA copying and repair. This can stop cancer cells dividing. Normal blood-forming cells can also be affected, and kidney function determines how the medicine is cleared.

Platinum reaches cells → DNA cross-links form → Copying and repair are disrupted

What to watch for

Contact the oncology emergency service immediately for fever or shaking chills, sudden illness, bleeding, new breathlessness, chest pain or severe allergy. Report reduced urine, hearing changes and new numbness promptly. Do not wait for the next visit.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The treatment being discussed

Breast treatment context

Advanced breast cancer. This patient suite describes the breast setting; the source protocol also has other tumour indications with their own clinical context.

Schedule and duration

Carboplatin is infused on Day 1 of each 21-day cycle. The team selects the dose using the prescribed AUC and kidney function. Treatment continues while helpful and tolerated; there is no fixed breast course length.

How treatment is given

Carboplatin is given through a vein over about 30 minutes after blood and symptom review and prescribed anti-sickness medicines. The complete appointment takes longer. A cannula or existing line may be used; a port is not automatically required.

Off-label breast indication

Carboplatin for metastatic breast cancer is an off-label use under this Irish NCCP protocol. The treating team must explain this, obtain informed consent and follow local Drugs and Therapeutics Committee approval requirements.

Why this treatment may be chosen

In the TNT study of advanced triple-negative or inherited BRCA1/2-associated breast cancer, carboplatin was active, but did not outperform docetaxel in the full study population. The inherited BRCA1/2 subgroup responded more often to carboplatin. These findings help inform a treatment discussion; they do not predict an individual result.

The team considers previous treatment, cancer features, health and your priorities. A response in a trial cannot guarantee benefit for an individual.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The phases and how they connect

Carboplatin · 21-day breast schedule

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY REFERENCE

Carboplatin · 21-day breast schedule

21-day interval

Carboplatin is infused on Day 1 of each 21-day cycle. The team selects the dose using the prescribed AUC and kidney function. Treatment continues while helpful and tolerated; there is no fixed breast course length.

These are the medicines in this exact schedule. Dose holds, missed tablets and omitted visits need instructions from your treating team.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Blood count, kidney and liver tests are checked before each cycle. The team confirms the kidney-function method, current AUC and previous dose changes. Report numbness, hearing changes, infection and bleeding. Additional tests follow symptoms and the individual plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00261 v7 · official regimen · Current Irish product information · safety supplement

THE WHOLE TREATMENT JOURNEY

Carboplatin · 21-day breast schedule: treatment and checks

Advanced breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00261	Confirm breast plan and current dose	Day 1 every 21 days	Continue while helpful and tolerated	Review dose and recovery before restart	Agree continuation / next treatment

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver NCCP 00261 · SMPC	Baseline blood and organ tests	Before cycle; extra tests if needed	Review blood tests and renal dosing each cycle	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines NCCP 00261 · SMPC	Platinum allergy / hearing / nerve review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check NCCP 00261 · SMPC	Confirm AUC and kidney method	Renal tests and prescribed antiemetics	21-day review pattern	Keep previous dose / nadir history	Personal plan; no booked dates implied

What each check is for

1

Treatment and dose review

Before every administration

Blood count, kidney and liver tests are checked before each cycle. The team confirms the kidney-function method, current AUC and previous dose changes. Report numbness, hearing changes, infection and bleeding. Additional tests follow symptoms and the individual plan.

Ask / act: The treating team confirms results, dose and suitability; this map does not authorise treatment.

2

Support and symptoms

Throughout treatment, including days at home

Contact the oncology emergency service immediately for fever or shaking chills, sudden illness, bleeding, new breathlessness, chest pain or severe allergy. Report reduced urine, hearing changes and new numbness promptly. Do not wait for the next visit.

Ask / act: Keep a written medicine plan and urgent-contact route.

3

Response and continuation

At agreed clinical review points

Aim to slow or shrink metastatic breast cancer and help symptoms, balancing possible benefit, side effects and treatment burden.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Visits and tests

One infusion visit per 21-day cycle plus pre-cycle blood tests and clinical review. Additional tests depend on symptoms and the individual plan.

STEP 2

Treatment preparation

Your team chooses venous access. A port is optional, requires a separate insertion procedure and later removal; it is not removed after every infusion.

STEP 3

At home

Allow time for blood review, preparation, anti-sickness treatment, the infusion and observation. No take-home infusion pump is part of this regimen.

STEP 4

Contacts / follow-up

Keep emergency contacts, medicine lists and the next appointment plan. Report line redness, swelling, leakage, pain or fever promptly.

Checks before and during treatment

Every dose depends on the reviewed treatment plan and results.

Blood and symptom checks

Heart and response checks

Kidney dosing, hearing and nerve review are central. Scans follow the consultant plan; no universal heart-scan schedule is specified.

Fertility, pregnancy and breastfeeding

Discuss fertility preservation before treatment if relevant. Carboplatin may affect periods and fertility, sometimes permanently. Do not breastfeed during treatment. Discuss reliable contraception and contact the team immediately if pregnancy is possible. The linked Irish SmPC advises effective contraception for women during treatment and at least 6 months afterwards; men should avoid fathering a child during treatment and at least 3 months afterwards. Confirm the actual product/local advice and any longer requirement from other medicines. Discuss low blood counts and sepsis, bleeding, nausea/vomiting, fatigue, kidney, hearing and nerve effects, and infusion allergy. Benefit is uncertain and alternatives depend on your cancer and priorities.

Timing the kidney check

If the team calculates kidney function from a blood creatinine result, the NCCP plan requires that result within 48 hours before carboplatin. If a measured kidney scan is used, a rise in creatinine can require a new dose calculation or kidney measurement. The team checks the method, trend and prescribed dose; do not calculate or change the dose yourself.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Carboplatin · 21-day breast schedule

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
IV treatment · Day 1	 **Day 1** Only after team clearance
Blood counts / infection	 **Days 1–21** Fever or sudden illness needs urgent help on any day
Nausea / bowel symptoms	 **Days 1–21** Use the prescribed support plan
Nerve / fatigue symptoms	 **Days 1–21** Report new or worsening symptoms early

Treatment and recovery: Carboplatin is infused on Day 1 of each 21-day cycle. The team selects the dose using the prescribed AUC and kidney function. Treatment continues while helpful and tolerated; there is no fixed breast course length.

Tests and decisions: Blood count, kidney and liver tests are checked before each cycle. The team confirms the kidney-function method, current AUC and previous dose changes. Report numbness, hearing changes, infection and bleeding. Additional tests follow symptoms and the individual plan.

Why these bands stay broad: This is a relative reference, not an appointment calendar. Treatment days can change after clinical review. Symptom bands do not predict when an effect will occur. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00261 v7 · official regimen ↗

There are no guaranteed "safe days". A day without treatment is not automatically a safe day. Use the emergency contact when needed; do not move or restart doses yourself.

Between-treatment notes

Day 1–21: One reference cycle

Carboplatin is infused on Day 1 of each 21-day cycle. The team selects the dose using the prescribed AUC and kidney function. Treatment continues while helpful and tolerated; there is no fixed breast course length. Follow the written plan from your treatment team; no dates are entered here.

Support around the medicines

1 Preventing sickness

Take the prescribed anti-sickness medicines before treatment and on the following days, even if you feel well. Follow your own prescription and contact the team if nausea or vomiting is not controlled.

2 Blood counts

Keep blood tests and use the emergency contact for fever, chills or bleeding. Preventive growth-factor injections depend on individual risk.

3 Kidneys and other medicines

Tell the team about every medicine or supplement, especially treatments that affect kidneys, hearing or bleeding. Follow individual drinking advice.

4 Steroid and rescue plan

The anti-sickness plan may affect sleep, glucose and bowel symptoms. Confirm the exact doses and last day of each medicine; contact the team if symptoms persist.

Support and safety at home

Keep the written medicine and urgent-contact plan.

Blood counts

Kidneys and other medicines

Questions worth asking

Is everyone given the same dose?

No. The consultant selects an AUC target within the source range and the team uses verified kidney function and previous effects to calculate the dose. Do not calculate or change it yourself.

How many cycles will I have?

The breast source continues until cancer progression or unacceptable toxicity. The team agrees response reviews and continuation; the fixed course of a particular trial or another cancer type does not set your course.

Do I need heart scans?

This carboplatin monotherapy regimen does not mandate routine heart scans. Tests are arranged when symptoms, other medicines or your individual risks make them appropriate.

> **Try explaining the plan back:** Which treatment days and doses are prescribed, which blood tests must happen first, how do you take the support medicines, and which symptoms need urgent help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Carboplatin AUC4–6 · 21-day schedule

Active medicines: Carboplatin

Emetogenic risk: High at AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally on Day 1; 80 mg orally on Days 2 and 3	Once daily on each listed day Check interactions; follow the prescribed plan.
Ondansetron	16 mg orally on Day 1	Before treatment NCIS permits individual splitting to 8 mg twice daily or reduction to 8 mg once daily. Review QT risk, electrolytes and constipation.
Dexamethasone	12 mg orally on Day 1; 8 mg orally on Days 2, 3 and 4	Once daily on each listed day Confirm timing and diabetes/sleep advice. This is the dedicated carboplatin row.
Metoclopramide · duration requires prescription	10 mg orally three times daily on subsequent days in the NCIS row	The source row does not specify an end day or PRN qualifier Prescriber/pharmacist must specify duration and suitability, including renal function and neurological contraindications. Do not continue indefinitely.

Additional phase-specific support

- NCIS Table 1 allows clinician-selected ondansetron splitting to 8 mg twice daily or reduction to 8 mg once daily according to the individual patient.
- Review ondansetron QT risk and other QT-prolonging medicines/electrolytes.
- Check dexamethasone interactions, diabetes, sleep and infection effects; aprepitant/steroid interaction is already reflected in the specified row.
- Review metoclopramide renal function, neurological contraindications, duration and extrapyramidal risk; do not automatically stack with olanzapine.
- AUC reduction below 4 requires an individual prophylaxis review; history of emesis and patient factors may justify continuing stronger protection.
- No universal granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or acid-suppression prescription is stated by the carboplatin protocol. G-CSF requires individual febrile-neutropenia risk review.
- Routine hypersensitivity premedication is usually unnecessary, but antiemetic prophylaxis is still required. Prior allergy/desensitisation is a separate specialist plan.
- Use the dedicated carboplatin AUC ≥4 row. It does not include routine olanzapine.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Blood / infection

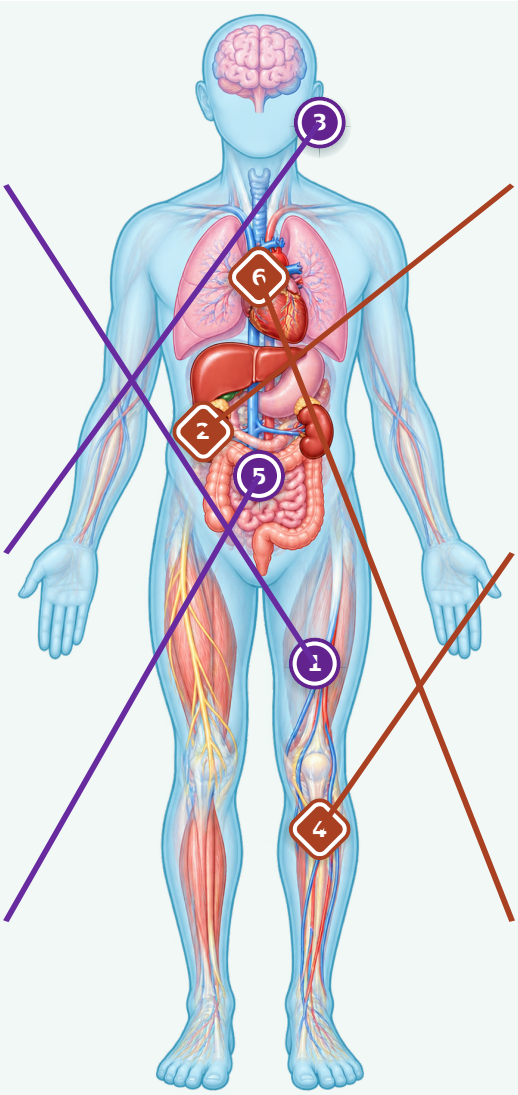
Carboplatin

Fever, unusual bruising or bleeding needs urgent advice.

2 Kidneys / urine

Carboplatin

Keep kidney tests and report reduced urine or swelling.



3 Hearing

Carboplatin

Report ringing in the ears or hearing changes.

4 Hands / feet

Carboplatin

Report tingling, numbness or altered balance early.

5 Nausea / appetite

Carboplatin

Follow the prevention plan; call if vomiting prevents fluids.

6 Allergic reaction

Carboplatin

During an infusion, report rash, chest tightness or breathlessness immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology emergency service immediately for fever or shaking chills, sudden illness, bleeding, new breathlessness, chest pain or severe allergy. Report reduced urine, hearing changes and new numbness promptly. Do not wait for the next visit.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00261 v7 · official regimen**
   https://healthservice.hse.ie/documents/6366/261_CARBOplatinAUC4-6_21days.pdf
2. **Current Irish product information · safety supplement**
   https://assets.hpra.ie/products/Human/28088/Licence_PA2059-032-001_02072024153224.pdf
3. **NCCP antiemetic guidance v6**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS antiemetic medicines v8 · dedicated carboplatin AUC ≥4 row, pages 4–5**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **Tutt et al. Carboplatin in BRCA1/2-mutated and triple-negative breast cancer BRCAness subgroups: the TNT Trial. Nature Medicine 2018;24:628–637**
   https://pubmed.ncbi.nlm.nih.gov/29713086/